

Influenza at the human-animal interface

Summary and risk assessment, from 9 May to 12 June 2026¹

- **New human cases²:** From 9 May to 12 June 2026, based on reporting date, detections of influenza A(H9N2) in four humans were reported officially.
- **Circulation of influenza viruses with zoonotic potential in animals:** High pathogenicity avian influenza (HPAI) events in poultry and non-poultry animal species continue to be reported to the World Organisation for Animal Health (WOAH).³ The Food and Agriculture Organization of the United Nations (FAO) also provides a global update on avian influenza viruses with pandemic potential.⁴ Additionally, low pathogenicity avian influenza viruses as well as swine influenza viruses continue to circulate in animal populations.
- **Risk assessment⁵:** Sustained human to human transmission has not been reported associated with the above-mentioned human infection events. Based on information available at the time of this risk assessment update, the overall public health risk from currently known influenza A viruses detected at the human-animal interface has not changed and remains low. At present, these viruses are not thought to be capable of sustained human-to-human transmission, although this could change as they evolve. Although human infections with viruses of animal origin are infrequent, they are not unexpected at the human-animal interface.
- **IHR compliance⁶:** This includes any influenza A virus that has demonstrated the capacity to infect a human and its haemagglutinin (HA) gene (or protein) is not a mutated form of those, i.e. A(H1) or A(H3), circulating widely in the human population. Information from these notifications is critical to inform risk assessments for influenza at the human-animal interface.

Avian influenza viruses in humans

A(H9N2), China

Between 13 May and 11 June 2026, China notified WHO of four laboratory-confirmed cases of A(H9N2) virus infection detected through influenza-like illness surveillance. All cases recovered from illness.

¹ This summary and assessment covers information confirmed during this period and may include information received outside of this period.

² For epidemiological and virological features of human infections with animal influenza viruses not reported in this assessment, see the reports on human cases of influenza at the human-animal interface published in the Weekly Epidemiological Record [here](#).

³ World Organisation for Animal Health (WOAH). Avian influenza. Global situation. Available at: <https://www.woah.org/en/disease/avian-influenza/#ui-id-2>.

⁴ Food and Agriculture Organization of the United Nations (FAO). Global Avian Influenza Viruses with Zoonotic Potential situation update. Available at: <https://www.fao.org/animal-health/situation-updates/global-aiv-with-zoonotic-potential>.

⁵ World Health Organization (2012). Rapid risk assessment of acute public health events. World Health Organization. Available at: <https://iris.who.int/handle/10665/70810>.

⁶ World Health Organization. Case definitions for the four diseases requiring notification in all circumstances under the International Health Regulations (2005). Available at: [https://www.who.int/publications/m/item/case-definitions-for-the-four-diseases-requiring-notification-to-who-in-all-circumstances-under-the-ihl-\(2005\)](https://www.who.int/publications/m/item/case-definitions-for-the-four-diseases-requiring-notification-to-who-in-all-circumstances-under-the-ihl-(2005)).

Age and sex	Province	Symptom onset
< 5-year-old male	Yunnan	28 April 2026
>60-year-old female	Guangxi	4 May 2026
< 5-year-old male	Yunnan	17 May 2026
< 5-year-old female	Guangxi	31 May 2026

The child with onset in April was admitted to hospital with pneumonia. The adult case had comorbidities and was hospitalized. All the cases had exposure to live bird markets or household poultry. Samples from environments associated with the likely area of exposure of all but one of these cases tested positive for A(H9) viruses. No further cases were detected among contacts of these cases.

Risk assessment for avian influenza A(H9N2):

1. What is the global public health risk of additional human cases of infection with avian influenza A(H9N2) viruses?

Most human cases follow exposure to the A(H9N2) virus through contact with infected poultry or contaminated environments. Most human infections of A(H9N2) to date have resulted in mild clinical illness. Since the virus is endemic in poultry in multiple countries in Africa and Asia, additional human cases associated with exposure to infected poultry or contaminated environments are expected but remain unusual. The impact to public health if additional sporadic cases are detected is minimal. The overall global public health risk is low.

2. What is the likelihood of sustained human-to-human transmission of avian influenza A(H9N2) viruses related to these events?

At the present time, no sustained human-to-human transmission has been identified associated with the recently reported human infections with A(H9N2) viruses. Current evidence suggests that A(H9N2) viruses from these cases did not acquire the ability of sustained transmission among humans.

3. What is the likelihood of international spread of avian influenza A(H9N2) virus by travellers?

Should infected individuals from affected areas travel internationally, their infection may be detected in another country during travel or after arrival. If this were to occur, further community level spread is considered unlikely as current evidence suggests the A(H9N2) virus subtype has not acquired the ability to transmit easily among humans.

Overall risk management recommendations:

Surveillance and investigations

- Due to the constantly evolving nature of influenza viruses, WHO continues to stress the importance of global strategic surveillance in animals and humans to detect virologic, epidemiologic and clinical changes associated with circulating influenza viruses that may affect human (or animal) health. Continued vigilance is needed within affected and neighbouring areas to detect infections in animals and humans. Close collaboration with the animal health and environment sectors is essential to understand the extent of the risk of human exposure and to prevent and control the spread of animal influenza. WHO has published guidance on [surveillance for human infections with avian influenza A\(H5\) viruses](#).
- As the extent of influenza virus circulation in animals is not clear, epidemiologic and virologic surveillance and the follow-up of suspected human cases should continue systematically. [Guidance on investigation of non-seasonal influenza and other emerging acute respiratory diseases](#) has been published on the WHO website.

- Countries should increase avian influenza surveillance in domestic and wild birds, enhance surveillance for early detection in cattle populations in countries where HPAI is known to be circulating, include HPAI as a differential diagnosis in non-avian species, including cattle and other livestock populations, with high risk of exposure to HPAI viruses; monitor and investigate cases in non-avian species, including livestock, report cases of HPAI in all animal species, including unusual hosts, to WOA and other international organizations, share genetic sequences of avian influenza viruses in publicly available databases, implement preventive and early response measures to break the HPAI transmission cycle among animals through movement restrictions of infected livestock holdings and strict biosecurity measures in all holdings, employ good production and hygiene practices when handling animal products, and protect persons in contact with suspected/infected animals.⁷ More guidance can be found from [WOAH](#) and [FAO](#).
- When there has been human exposure to a known outbreak of an influenza A virus in domestic poultry, wild birds or other animals – or when there has been an identified human case of infection with such a virus – enhanced surveillance in potentially exposed human populations becomes necessary. Enhanced surveillance should consider the health care seeking behaviour of the population, and could include a range of active and passive health care and/or community-based approaches, including: enhanced surveillance in local influenza-like illness (ILI)/SARI systems, active screening in hospitals and of groups that may be at higher occupational risk of exposure, and inclusion of other sources such as traditional healers, private practitioners and private diagnostic laboratories.
- Vigilance for the emergence of novel influenza viruses with pandemic potential should be maintained at all times including during a non-influenza emergency. In the context of the co-circulation of SARS-CoV-2 and influenza viruses, WHO has updated and published [practical guidance for integrated surveillance](#).

Notifying WHO

- All human infections caused by a new subtype of influenza virus are notifiable under the International Health Regulations (IHR, 2005).^{8,9} State Parties to the IHR (2005) are required to immediately notify WHO of any laboratory-confirmed¹⁰ case of a recent human infection caused by an influenza A virus with the potential to cause a pandemic¹¹. Evidence of illness is not required for this report. Evidence of illness is not required for this report.
- WHO published the case definition for human infections with avian influenza A(H5) virus requiring notification under IHR (2005): <https://www.who.int/teams/global-influenza-programme/avian-influenza/case-definitions>.

Virus sharing and risk assessment

- It is critical that these influenza viruses from animals or from humans are fully characterized in appropriate animal or human health influenza reference laboratories. Under WHO's Pandemic

⁷ World Organisation for Animal Health. Statement on High Pathogenicity Avian Influenza in Cattle, 6 December 2024 (<https://www.woah.org/en/high-pathogenicity-avian-influenza-hpai-in-cattle/>).

⁸ World Health Organization. International Health Regulations (2005), as amended through resolutions WHA67.13 (2014), WHA75.12 (2022), and WHA77.17 (2024) (https://apps.who.int/gb/bd/pdf_files/IHR_2014-2022-2024-en.pdf).

⁹ World Health Organization. Case definitions for the four diseases requiring notification in all circumstances under the International Health Regulations (2005) ([https://www.who.int/publications/m/item/case-definitions-for-the-four-diseases-requiring-notification-to-who-in-all-circumstances-under-the-ih-r-\(2005\)](https://www.who.int/publications/m/item/case-definitions-for-the-four-diseases-requiring-notification-to-who-in-all-circumstances-under-the-ih-r-(2005))).

¹⁰ World Health Organization. Manual for the laboratory diagnosis and virological surveillance of influenza (2011) (<https://apps.who.int/iris/handle/10665/44518>).

¹¹ World Health Organization. Pandemic influenza preparedness framework for the sharing of influenza viruses and access to vaccines and other benefits, 2nd edition (<https://iris.who.int/handle/10665/341850>).

Influenza Preparedness (PIP) Framework, Member States are expected to share influenza viruses with pandemic potential on a **timely basis**¹² with a WHO Collaborating Centre for influenza of GISRS. The viruses are used by the public health laboratories to assess the risk of pandemic influenza and to develop candidate vaccine viruses.

- The Tool for Influenza Pandemic Risk Assessment (TIPRA) provides an in-depth assessment of risk associated with some zoonotic influenza viruses – notably the likelihood of the virus gaining human-to-human transmissibility, and the impact should the virus gain such transmissibility. TIPRA maps relative risk amongst viruses assessed using multiple risk elements. The results of TIPRA complement those of the risk assessment provided here, and those of prior TIPRA risk assessments are published at [http://www.who.int/teams/global-influenza-programme/avian-influenza/tool-for-influenza-pandemic-risk-assessment-\(tipra\)](http://www.who.int/teams/global-influenza-programme/avian-influenza/tool-for-influenza-pandemic-risk-assessment-(tipra)).

Risk reduction

- Given the observed extent and frequency of avian influenza in poultry, wild birds and some wild and domestic mammals, the public should avoid contact with animals that are sick or dead from unknown causes, including wild animals, and should report dead birds and mammals or request their removal by contacting local wildlife or veterinary authorities.
- Eggs, poultry meat and other poultry food products should be properly cooked and properly handled during food preparation. Due to the potential health risks to consumers, raw milk should be avoided. WHO advises consuming pasteurized milk. If pasteurized milk isn't available, heating raw milk until it boils makes it safer for consumption.
- WHO has published [practical interim guidance to reduce the risk of infection in people exposed to avian influenza viruses](#).

Trade and travellers

- WHO advises that travellers to countries with known outbreaks of animal influenza should avoid farms, contact with animals in live animal markets, entering areas where animals may be slaughtered, or contact with any surfaces that appear to be contaminated with animal excreta. Travelers should also wash their hands often with soap and water. All individuals should follow good food safety and hygiene practices.
- WHO does not advise special traveller screening at points of entry or restrictions with regards to the current situation of influenza viruses at the human-animal interface. For recommendations on safe trade in animals and related products from countries affected by these influenza viruses, refer to [WOAH](#) guidance.

Links:

WHO Human-Animal Interface web page

<https://www.who.int/teams/global-influenza-programme/avian-influenza>

WHO Influenza (Avian and other zoonotic) fact sheet

[https://www.who.int/news-room/fact-sheets/detail/influenza-\(avian-and-other-zoonotic\)](https://www.who.int/news-room/fact-sheets/detail/influenza-(avian-and-other-zoonotic))

WHO Protocol to investigate non-seasonal influenza and other emerging acute respiratory diseases

<https://www.who.int/publications/i/item/WHO-WHE-IHM-GIP-2018.2>

WHO Public health resource pack for countries experiencing outbreaks of influenza in animals:

<https://www.who.int/publications/i/item/9789240076884>

Cumulative Number of Confirmed Human Cases of Avian Influenza A(H5N1) Reported to WHO

<https://www.who.int/teams/global-influenza-programme/avian-influenza/avian-a-h5n1-virus>

Avian Influenza A(H7N9) Information

¹² World Health Organization. Operational guidance on sharing influenza viruses with human pandemic potential (IVPP) under the Pandemic Influenza Preparedness (PIP) Framework (2017) (<https://apps.who.int/iris/handle/10665/259402>).

[https://www.who.int/teams/global-influenza-programme/avian-influenza/avian-influenza-a-\(h7n9\)-virus](https://www.who.int/teams/global-influenza-programme/avian-influenza/avian-influenza-a-(h7n9)-virus)

World Organisation of Animal Health (WOAH) web page: Avian Influenza

<https://www.woah.org/en/home/>

Food and Agriculture Organization of the United Nations (FAO) webpage: Avian Influenza

<https://www.fao.org/animal-health/avian-flu-qa/en/>

WOAH/FAO Network of Expertise on Animal Influenza (OFFLU)

<http://www.offlu.org/>